

Notes

Patient: K CHANDRIKA CHANDRIKA

DOB: 23/07/1970

UHID: KABHK.0000126087

Age: 54

Gender: Female

Type: Phys-OP Notes

21 May 2025 05:02:34 PM Dr.Satheesh C T:

Old case of GIST
 Diagnosed in 2011
 Underwent surgery
 No adjuvant Rx
 Rec GIST - 2013
 DOG1 + CKIT - positive
 Started Imatinib from 2013 - March 2021
 April 2021 PET CT - Progressive disease
 Later started Sunitinib - April 2021
 COMORBID- DM
 Symptomatically better
 Mucositis Grade 1
 HFS- Grade 2
 GI symptoms grade 1
 Nov 2021- PET CT s/o Good partial response
 Underwent Debulking surgery on 25.11.21
 HPE + IHC S/o Rec GIST
 CKIT Mutation s/o Insertion mutation in Exon9
 Later continued on Sunitinib but poor tolerance and April 2022 - Developed recurrent Disease
 hence started with Rogerafenib
 But patient is having fatigue
 USG abd pelvis- progressive disease
 29th Nov 2022: PET scan - S/o recurrent and progressive disease

Patient proxy

Comobid; Dm
 USG Guided Biopsy of abdominal MASS for HPE and (NGS - CGP) - Not done

SBRT completed by Dr Lohith on 13/10/23

PET SCAN (14 TH APRIL 2025) S/O -

Status post excision of left lower abdominal wall and intraabdominal lesions.

Re-demonstration of lobulated soft tissue lesions in the lower anterior abdominal wall and suprapubic region as described above.

Re-demonstration of multiple deposits involving omental, pelvis, peritoneum with extensions and infiltration as described above. Re-demonstration of focal areas of bowel infiltration and communication between the left omental deposit and jejunum, there is demonstration of extension of enteric contrast into the mass. Re-demonstration of urinary bladder and lateral pelvic wall infiltration.

Re-demonstration of mild bilateral hydronephrosis.

MDT DISCUSSION DONE WITH DR . PRABHU AND WITH DR . RAGHUNATH AND RO DR LOHITH PLAN FOR DJ STENTING AND DIVERSION PROCEDURE FOR INTESTINAL FISTULA WITH TISSUE SAMPLING .

PROGNOSIS , OPTIONS , COMPLICATIONS EXPLAINED TO PATIENT AND ATTENDERS

Notes

The patient underwent IVC FILTER PLACEMENT AND EMBOLIZATION OF THE PELVIC GIST by Dr. Indushekar on 05.05.2025

The patient underwent DIAGNOSTIC LAPAROSCOPY, LAPAROTOMY AND TRANSVERSE COLON LOOP COLOSTOMY and B/L URETERIC STENING under GA by Dr. Prabhu N Nesargikar and Dr Raghunath (urologist) on 06.05.2025.

HPE S/O - SPINDLE CELL NEOPLASM MORPHOLOGY FAVOURS GASTROINTESTINAL STROMAL TUMOR , SPINDLE CELL TYPE (16 TH MAY 2025)

ADVICE ;

TAB - CARTIQTEN 1-0-1 X DAILY AFTER FOOD

continue regular DM medicines

Tab cyclopam 1 sos for abdominal pain

Frequent diet as per advise

REVIEW AFTER 1 WEEK IN OPD WITH URINE CULTURE AND SENSITIVITY

SYRUP - CITRALKAL 2TSP - 2TSP - 2TSP WITH 1 GLASS OF WATER

TAB - NIFTAS 100 MG 1-0-1 X 5 DAYS AFTER FOOD

TAB - SHELICAL 500 MG 1-0-0 X DAILY AFTER FOOD

